

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 945

BY WAYS AND MEANS COMMITTEE

AN ACT

RELATING TO ARTIFICIAL INTELLIGENCE MEDICAL SERVICES; PROVIDING LEGISLATIVE FINDINGS AND INTENT; AMENDING TITLE 54, IDAHO CODE, BY THE ADDITION OF A NEW CHAPTER 60, TITLE 54, IDAHO CODE, TO PROVIDE A SHORT TITLE, TO DEFINE TERMS, TO ESTABLISH THE BOARD OF AUTONOMOUS MEDICAL PRACTICE, TO PROVIDE FOR A LICENSURE FRAMEWORK, TO PROVIDE FOR TRANSPARENCY, TO PROVIDE FOR CLINICAL INTEGRITY, PATIENT LOYALTY, AND AUDITABILITY, TO PROVIDE FOR A REGULATORY SANDBOX AND ETHICAL OVERSIGHT, TO PROVIDE FOR SCOPE OF PRACTICE AUTHORITY AND A LIABILITY WAIVER, TO PROVIDE FOR ENFORCEMENT, TO PROVIDE FOR STATE FUNDING, TO PROVIDE FOR INSURANCE GUIDANCE, AND TO PROVIDE A RULE OF CONSTRUCTION; PROVIDING SEVERABILITY; AND DECLARING AN EMERGENCY AND PROVIDING AN EFFECTIVE DATE.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. LEGISLATIVE FINDINGS AND INTENT. (1) The Legislature finds that:

(a) The patient-directed diagnostic, therapeutic, triaging, prescribing, or other clinical application of artificial intelligence, when offered as a clinical AI service, may constitute the practice of medicine or other licensed clinical practice subject to state regulation.

(b) Laboratory-developed tests and professional diagnostic services that are solely patient-specific professional services within this state, and not marketed or distributed as general purpose commercial products, are professional services subject to state regulation, distinct from the commercial sale of medical devices regulated by the federal food, drug, and cosmetic act.

(c) To address critical provider shortages and improve patient outcomes, the state must establish a clear, safe licensure pathway for autonomous and augmented clinical services delivered through software, systems, or devices while preserving state authority and avoiding conflict with applicable federal law.

(d) The regulation of autonomous clinical decision-making is demonstrably necessary to protect the life, health, and safety of the citizens of this state. The licensure requirements established in this act are narrowly tailored to regulate only the professional application of artificial intelligence in a clinical setting and do not restrict the general private ownership or nonclinical use of computational resources.

(e) The purpose of professional licensure and regulation is to protect the public health, safety, and welfare. Nothing in this act is intended to preserve or advance the economic interests, market share, or professional boundaries of any particular health care profession. This act shall be construed to promote patient access to safe and effective artificial intelligence services and to encourage responsible innovation.

Any regulatory requirement adopted pursuant to this act that materially affects market entry or the availability of AI-enabled services shall be reasonably related to, and proportionate with, an identified risk to patient safety and shall not impose restrictions greater than necessary to address such risk.

(f) Laboratory-developed tests and professional diagnostic services utilizing proprietary methodologies, when offered and performed solely as patient-specific professional services within this state and not marketed or distributed as general-purpose commercial products, constitute professional services subject to state regulation. Nothing in this act shall be construed to expand or limit the application of applicable federal law.

(g) Nothing in this act expands or restricts the scope of practice of any human-licensed health care profession; rather, this act creates an independent licensure pathway for clinical AI services.

(2) It is the intent of the Legislature through this act to create a new class of health care provider, the AI-augmented and autonomous service provider, capable of practicing medicine or other licensed clinical practice under state licensure.

SECTION 2. That Title 54, Idaho Code, be, and the same is hereby amended by the addition thereto of a NEW CHAPTER, to be known and designated as Chapter 60, Title 54, Idaho Code, and to read as follows:

CHAPTER 60 ARTIFICIAL INTELLIGENCE MEDICAL SERVICES

54-6001. SHORT TITLE. This chapter shall be known and may be cited as the "AI Medical Services Act."

54-6002. DEFINITIONS. For the purposes of this chapter:

(1) "AAASP" or "artificial intelligence augmented and autonomous service provider" means a corporate or legal entity licensed by the board to operate clinical AI services that are subject to licensure pursuant to section 54-6004, Idaho Code.

(a) For the purposes of state law governing patient privacy, confidentiality, credentialing, reimbursement, and professional accountability as administered under this chapter, an AAASP is deemed a health care provider.

(b) For the purposes of state-administered patient privacy, confidentiality, credentialing, and reimbursement laws, and for federal laws that expressly defer to state determinations of provider status, including the federal health insurance portability and accountability act (HIPAA), an AAASP is deemed a health care provider, without creating or implying recognition for purposes of medicare, medicaid, or other federal payment programs.

(c) An AAASP shall be treated as a health care provider under all applicable state privacy and confidentiality laws and shall comply with HIPAA to the extent it functions as a covered entity or business associate.

1 (2) "Adverse event" means an event of patient death, serious physical
2 or psychological harm, or serious risk of harm reasonably associated with a
3 clinical AI service, including inappropriate triage or failure to escalate
4 care.

5 (3) "AI" means artificial intelligence.

6 (4) AI operational role categories:

7 (a) "Advisory AI" means artificial intelligence that analyzes pa-
8 tient-specific data to generate options, potential diagnoses, risk
9 stratification, or therapeutic suggestions for a licensed health care
10 provider or directly for a patient or user, where such output is in-
11 tended to inform but not substitute for independent clinical judgment
12 and where the provider or user is expected to review, contextualize,
13 and determine whether and how to act regarding the suggestion for each
14 patient encounter.

15 (b) "Fully autonomous AI" means artificial intelligence authorized to
16 independently diagnose, treat, triage, or prescribe without the neces-
17 sity of human supervision or intervention for each distinct case.

18 (c) "Informational AI" means artificial intelligence that provides ag-
19 gregated data, literature, or administrative information to a user but
20 does not suggest a specific clinical action, such as search retrieval,
21 transcription, or back-office support.

22 (d) "Supervised autonomous AI" means artificial intelligence autho-
23 rized to generate and execute a clinical action, diagnosis, or treat-
24 ment plan under the supervision of a licensed human provider who retains
25 the ability to intervene.

26 (5) "Board" means the board of autonomous medical practice.

27 (6) (a) "Clinical AI service" means any software system, algorithmic
28 model, or automated service that, whether independently or in combina-
29 tion with human involvement, performs, supports, or materially influ-
30 ences functions that constitute the practice of medicine or other li-
31 censed clinical practice with respect to a specific patient, to the ex-
32 tent otherwise permitted under applicable law.

33 (b) "Clinical AI service" encompasses AI systems across a spectrum of
34 functionality and autonomy, including information-providing, advi-
35 sory, supervised autonomous, and autonomous systems, when such systems
36 engage in patient-directed clinical functions as described in this sub-
37 section. Such functions include but are not limited to:

38 (i) Collecting, analyzing, interpreting, or synthesizing clini-
39 cal data, biometric data, diagnostic data, laboratory data, imag-
40 ing data, or patient-reported information for a specific patient;

41 (ii) Generating or supporting clinical assessments, diagnostic
42 conclusions, risk stratification, triage determinations, treat-
43 ment plans, medication selection or dosing, or therapeutic initi-
44 ation, modification, continuation, or discontinuation for a spe-
45 cific patient;

46 (iii) Recommending, supporting, or interpreting laboratory
47 tests, diagnostic procedures, or monitoring parameters for a spe-
48 cific patient;

49 (iv) Monitoring patient status, detecting clinically meaning-
50 ful changes, predicting clinical deterioration or improvement,

1 or supporting clinical intervention workflows for a specific pa-
2 tient, whether in real time or asynchronously; and

3 (v) Communicating clinical information, recommendations,
4 alerts, or guidance intended to inform patient care or licensed
5 clinician decision-making for a specific patient.

6 (c) "Clinical AI service" may be deployed through any care delivery
7 modality, including in-person clinical systems, or software-based
8 agents. The mode of deployment does not alter whether a system consti-
9 tutes a clinical AI service under this definition.

10 (d) "Clinical AI service" does not include software or systems whose
11 sole function is administrative, financial, scheduling, billing, in-
12 ventory, claims processing, documentation assistance, generalized
13 clinical education, population-level analytics, research, quality
14 improvement, public health surveillance, or de-identified data pro-
15 cessing, provided such systems do not materially influence clinical
16 decision-making for a specific patient.

17 (e) "Clinical AI service" also does not include purely informational
18 systems that only transmit, store, display, or route data without per-
19 forming clinical analysis, interpretation, or decision support, unless
20 such systems materially influence patient-specific clinical functions
21 described in paragraph (b) of this subsection. A system is considered
22 deployed as a clinical AI service if it is marketed, represented, or
23 reasonably used to independently perform patient-specific clinical
24 decision-making or autonomous clinical action. A system is not consid-
25 ered deployed as a clinical AI service when it merely provides advisory
26 clinical decision support to a licensed human health care provider who
27 retains independent authority for clinical decisions and execution.

28 (7) (a) Clinical condition categories:

29 (i) "Chronic condition" means a human disease, disorder, in-
30 jury, or impairment that is persistent, recurrent, or reasonably
31 expected to require ongoing or periodic clinical management,
32 monitoring, or care to maintain function, prevent progression,
33 mitigate symptoms, or reduce the risk of complications. Chronic
34 conditions are primarily managed through longitudinal care rather
35 than isolated emergency intervention and may experience episodic
36 exacerbations requiring temporary escalation of care.

37 (ii) "Critical condition" means a disease, illness, injury, or
38 physiologic state in which one (1) or more vital organ systems is
39 impaired, failing, or at substantial risk of failure, or in which
40 the condition presents a high probability of death, permanent dis-
41 ability, or serious irreversible harm without prompt and advanced
42 clinical intervention. A critical condition is characterized by
43 physiologic instability, high acuity, or the need for continuous
44 monitoring, specialized resources, or intensive medical manage-
45 ment to prevent catastrophic outcomes.

46 (iii) "Noncritical condition" means a condition, illness, or in-
47 jury, whether acute, subacute, stable chronic, or self-limiting,
48 for which, based on reasonable clinical judgment and available
49 clinical information, a delay in definitive diagnosis, initia-
50 tion of treatment, or escalation of care would not reasonably be

1 expected to result in serious adverse health consequences, per-
2 manent disability, or death. A noncritical condition does not
3 present objective signs of physiologic instability, rapidly pro-
4 gressive deterioration, or the need for immediate emergency or
5 life-preserving intervention.

6 (iv) "Preventive condition" means a medical condition where
7 measures, services, or interventions are applied to reduce the
8 likelihood of disease onset, progression, recurrence, or compli-
9 cations, including wellness, fitness, primary, and risk-based
10 preventive care, whether applied to healthy individuals or in-
11 dividuals with identified risk factors or existing conditions,
12 where the primary purpose is risk reduction or health maintenance
13 rather than treatment of an active acute pathology and where the
14 intervention is generally low-risk and consistent with accepted
15 standards of care.

16 (v) "Time-sensitive condition" means a medical condition or
17 acute clinical presentation for which the effectiveness of di-
18 agnosis, treatment, or intervention is materially dependent on
19 timely initiation and for which a delay in care is reasonably ex-
20 pected to result in rapid clinical deterioration, irreversible
21 morbidity, or death. Time-sensitive conditions require acceler-
22 ated recognition, triage, and escalation of care based on reason-
23 able clinical judgment and available clinical information.

24 (b) Clinical condition categorization under this chapter shall be
25 based on the patient's presentation and the clinical information rea-
26 sonably available at the time of the service, not on retrospective
27 diagnosis. A licensee acting in reasonable reliance on available clin-
28 ical information shall not be deemed in violation of the provisions of
29 the chapter solely due to later reclassification of the condition.

30 (8) "Designated responsible official" means a natural person desig-
31 nated by an AAASP who is authorized to bind the AAASP for compliance and
32 administrative matters under this chapter, to receive legal process and
33 board notices, and to certify filings and reports required by the board. A
34 designated responsible official is not, solely by designation, deemed to be
35 practicing a licensed health care profession.

36 (9) "Materially influence" means having a reasonable likelihood of be-
37 ing relied on to make, modify, or forego a patient-specific clinical deci-
38 sion or action.

39 (10) "Medical director" means a physician licensed and in good stand-
40 ing in this state who is designated by an AAASP to provide clinical oversight
41 of the AAASP's clinical scope, safety protocols, escalation pathways, and
42 quality assurance processes, as required by this chapter or board rule.

43 (11) "Reportable event" means an adverse event, a material near miss,
44 a material malfunction affecting clinical output, or a material data in-
45 tegrity failure affecting patient-specific clinical decisions.

46 (12) "Sandbox reciprocity state" means a jurisdiction recognized by the
47 board as having a substantially similar regulatory sandbox for health tech-
48 nology.

1 54-6003. BOARD OF AUTONOMOUS MEDICAL PRACTICE. (1) There is hereby
 2 established the board of autonomous medical practice in the division of oc-
 3 cupational and professional licenses. The board's jurisdiction extends to
 4 clinical AI services constituting the practice of medicine or other licensed
 5 clinical practice performing clinical AI services as defined in section
 6 54-6002, Idaho Code.

7 (2) The board shall consist of eleven (11) voting members appointed by
 8 and serving at the pleasure of the governor. Members may be removed by the
 9 governor at any time, with or without cause. The board shall include:

- 10 (a) One (1) licensed medical doctor or doctor of osteopathy;
- 11 (b) One (1) registered nurse or advanced practice registered nurse;
- 12 (c) One (1) licensed pharmacist;
- 13 (d) One (1) licensed mental or behavioral health professional;
- 14 (e) One (1) member representing hospitals, such as a hospital associa-
 15 tion employee or hospital chief executive officer;
- 16 (f) One (1) health care ethicist with an advanced degree or significant
 17 professional experience in medical ethics or bioethics;
- 18 (g) One (1) public member representing the interests and protections of
 19 consumers with respect to clinical AI services; and
- 20 (h) Four (4) members at large with demonstrated expertise in any of the
 21 following fields: health technology, artificial intelligence, systems
 22 engineering, health care administration, patient safety, or regulatory
 23 affairs.

24 (3) (a) The division administrator shall appoint an executive officer
 25 who shall serve as the chief administrative officer of the board and
 26 shall be responsible for the administration and operation of the board.
 27 The executive officer's compensation shall be funded through state
 28 appropriations. The executive officer shall be considered a nonclassi-
 29 fied employee, an executive employee, and an exempt employee.

30 (b) The division administrator or executive officer may employ such
 31 other staff as necessary to carry out the duties of the board, includ-
 32 ing a board ethicist as required by section 54-6007(4), Idaho Code, who
 33 shall be a nonvoting member of the board.

34 (c) Except as otherwise expressly reserved to the board by statute or
 35 rule, the division administrator or executive officer is authorized to
 36 take all actions reasonably necessary to carry out and enforce the laws
 37 and rules administered by the board, including but not limited to the
 38 authority to:

- 39 (i) Employ, supervise, evaluate, and discipline staff and con-
 40 tractors;
- 41 (ii) Enter into contracts, procure goods and services, and make
 42 expenditures within appropriated or authorized budgets;
- 43 (iii) Establish internal organizational structure, operational
 44 procedures, and administrative systems;
- 45 (iv) Receive, process, investigate, and resolve applications,
 46 registrations, filings, complaints, audits, and compliance mat-
 47 ters;
- 48 (v) Conduct investigations, issue requests for information, and
 49 require the production of records as authorized by law;

(vi) Administer examinations, reviews, assessments, certifications, or registrations authorized by law or rule; and

(vii) Take any other administrative or operational actions necessary to efficiently carry out the purposes of this chapter.

(4) (a) The board is authorized to adopt and charge reasonable fees for applications, provisional licenses, license renewals, and other administrative services. Fees shall be set at a level sufficient to offset the administrative costs of the regulatory sandbox and licensure monitoring.

(b) The legislature may appropriate funds as necessary for the initial establishment and operation of the board. Any such appropriation shall be temporary and shall terminate once fee revenue generated under this section is sufficient to support the ongoing administrative costs of the board.

(5) Members shall serve staggered four (4) year terms. The governor shall make appointments to promote regular turnover and avoid regulatory capture. No member may serve more than two (2) consecutive terms.

(6) The board shall:

(a) Monitor and have the right to grant, suspend, and revoke AAASP licenses of all classes and types;

(b) Establish and operate or contract for a state centralized institutional review board;

(c) Authorize and develop frameworks for delegated agreements, collaborative practice agreements, and supervision agreements, collectively referred to as clinical practice agreements in this chapter;

(d) Conduct or contract for algorithmic safety and bias audits; and

(e) Issue unique state provider identifiers for billing.

(7) The board may adopt rules in accordance with the provisions of chapter 52, title 67, Idaho Code:

(a) As necessary to administer, implement, and enforce the provisions of this chapter consistent with legislative intent and the authority expressly granted to the board, including rules governing licensure standards, application procedures, renewal requirements, recordkeeping, reporting, inspections, audits, and compliance oversight;

(b) To establish minimum standards of professional conduct, operational compliance, and public protection applicable to licensees, within the scope and purposes of this chapter; and

(c) To prescribe fees, forms, and administrative processes necessary to carry out the duties of the board.

(8) (a) The board shall meet at least quarterly and at such additional times as may be necessary to carry out the duties of the board.

(b) A meeting of the board may be called by the chair, the executive officer, or upon written request of a majority of the board members.

(c) The board shall annually elect from among its members a chair and vice chair, who shall serve one (1) year terms in such positions and may be reelected.

(d) A majority of the voting members of the board shall constitute a quorum for the transaction of business. An affirmative vote of a majority of the members present at a meeting at which a quorum is present

shall be required for official action of the board, unless otherwise provided by law.

(e) All meetings of the board shall be open to the public and conducted in compliance with the state open meetings law and public records act. The board may enter executive session only as authorized by law.

(f) The board shall permit public participation in meetings by teleconference or other electronic means, provided that public access and quorum requirements are satisfied.

(g) No rule that materially restricts AAASP scope of practice or imposes a new material barrier to market entry shall be adopted without:

(i) At least a two-thirds (2/3) affirmative vote of the board; and

(ii) Written findings that the restriction is supported by substantial evidence of a patient-safety risk and is the least restrictive means to address such risk.

(9) As the specific licensing authority for AAASPs, the board:

(a) May receive complaints, conduct investigations, require the production of records reasonably related to compliance with this chapter, and conduct audits and inspections as authorized by law and rule;

(b) May issue administrative subpoenas for testimony and documents in furtherance of an investigation or contested case;

(c) May impose discipline, including reprimand, probation, restricted licensure, suspension, revocation, and administrative fines, if an AAASP fails to meet the applicable standard of care or violates this chapter or board rules; and

(d) Must conduct any denial, discipline, or adverse licensure action as a contested case subject to the judicial review provisions of chapter 52, title 67, Idaho Code.

54-6004. LICENSURE FRAMEWORK. (1) The following AI operational categories are exempt from licensure under this chapter, provided they do not exceed the scope set forth in this section:

(a) All informational AI systems, regardless of the clinical condition they are applied to;

(b) Advisory AI as applied to preventive, chronic, or noncritical conditions; and

(c) Supervised autonomous AI applied strictly to preventive conditions and not issuing patient-specific clinical orders as part of a licensed professional service rendered within this state, including but not limited to medication orders, laboratory orders, or device orders.

(2) The board shall issue AAASP licenses with a base class and an autonomy modifier, as provided in this subsection.

(a) Base classes:

(i) Class A, state clinical service, is for clinical AI services delivered as patient-specific professional services and regulated by the state pursuant to its authority over the practice of medicine or other licensed clinical practice, including but not limited to services operating in a manner analogous to laboratory-developed tests or other proprietary algorithmic diagnostic, triaging, or therapeutic services, that do not rely on

1 food and drug administration clearance or approval as the basis
2 for their lawful clinical use.

3 (ii) Class B, federal device reciprocity service, is for clinical
4 AI services that have achieved food and drug administration clear-
5 ance, authorization, or approval as software as a medical device
6 and for which such federal authorization serves as the primary ba-
7 sis for the system's lawful clinical use.

8 (iii) Class C, therapeutic and support service, is for clinical AI
9 services providing nondiagnostic therapy, coaching, or monitor-
10 ing. Class C services do not independently establish a diagnosis
11 but operate on the basis of an existing diagnosis, referral, or pa-
12 tient-identified condition.

13 (b) Autonomy modifiers:

14 (i) Modifier L0, voluntary or exempt, is for AI systems otherwise
15 exempt from licensure under subsection (1) of this section that
16 voluntarily elect to obtain licensure.

17 (ii) Modifier L1, advisory-critical, is for advisory AI address-
18 ing critical or time-sensitive conditions.

19 (iii) Modifier L2, supervised, is for supervised autonomous AI re-
20 quiring human oversight or clinical practice agreements.

21 (iv) Modifier L3, autonomous, is for fully autonomous AI autho-
22 rized for independent operation.

23 (3) (a) In order to protect clinical decision support from liability,
24 an advisory AI system shall not require licensure under this chapter,
25 including under modifier L1, when the system:

26 (i) Does not independently initiate, execute, modify, or discon-
27 tinue a clinical action, order, diagnosis, or treatment; and

28 (ii) Is not intended, represented, or reasonably relied on as a
29 substitute for independent professional clinical judgment in the
30 management of a critical or time-sensitive condition.

31 (b) Nothing in this subsection exempts any system that otherwise meets
32 the definition of supervised autonomous AI or fully autonomous AI.

33 (4) An AAASP license is required for any entity operating clinical AI
34 services that fall within the following categories:

35 (a) Critical advisory AI: Advisory AI tools applied to critical and
36 time-sensitive conditions that are intended, represented, or reason-
37 ably relied on to guide clinical action in a manner that substitutes
38 for, rather than merely informs, independent clinical judgment and that
39 does not satisfy the clinical decision support liability protections
40 set forth in subsection (3) of this section.

41 (b) Clinical supervised AI: Supervised autonomous AI applied to
42 chronic, noncritical, critical, or time-sensitive conditions, or
43 preventive conditions but only where the service includes patient-spe-
44 cific clinical orders, including but not limited to medication orders,
45 laboratory orders, or device orders as part of a licensed professional
46 service rendered within this state.

47 (c) Fully autonomous AI: All fully autonomous AI systems, regardless of
48 the condition category they are applied to.

49 (5) Licenses granted pursuant to this chapter may be:

1 (a) A provisional, or sandbox, AAASP license, which shall be the ini-
2 tial license issued to all new applicants, that restricts the licensee
3 to operating within the regulatory sandbox subject to the oversight,
4 geographic limitations, and data-reporting requirements of section
5 54-6007, Idaho Code; or

6 (b) A full, or unrestricted, AAASP license, which shall be the standard
7 license permitting statewide practice, issued upon successful comple-
8 tion from the sandbox.

9 (6) Licensed human providers utilizing exempt AI tools within their
10 standard scope of practice are not required to obtain an AAASP license.
11 However, an exempt AI entity may voluntarily apply for AAASP licensure des-
12 ignated as "modifier L0" to obtain standalone reimbursement or enter into
13 clinical practice agreements.

14 (7) A licensee may obtain license reciprocity by sandbox reciprocity,
15 federal reciprocity, or licensure by recognition in substantially similar
16 jurisdictions, as provided in this subsection.

17 (a) A licensee in good standing in a sandbox reciprocity state, and not
18 the subject of any active or pending disciplinary action, shall be eli-
19 gible for licensure by recognition in the Idaho state sandbox upon sub-
20 mission of a completed application.

21 (b) An entity holding a valid federal clearance for the specific use
22 case applied for shall be automatically eligible for class B licensure.
23 The board may impose state conditions of licensure under this chapter,
24 including transparency, reporting, auditing, pilot-zone, and sandbox
25 requirements, to the extent not inconsistent with federal law, provided
26 that they align with benchmarks, post-market monitoring plans, and re-
27 lated guidelines already applicable to that provider to the maximum ex-
28 tent practicable.

29 (c) (i) The board shall grant an AAASP license to an applicant
30 holding a current, unrestricted authorization to provide substan-
31 tially similar clinical AI services in another state, unless the
32 board determines that:

33 1. The originating jurisdiction's regulatory framework is
34 materially less protective of patient safety than this chap-
35 ter;

36 2. The applicant is not in good standing or is the subject of
37 pending disciplinary action; or

38 3. The scope of practice or autonomy level requested in this
39 state exceeds that authorized in the originating jurisdic-
40 tion.

41 (ii) Licensure granted under this paragraph shall not require
42 satisfaction of initial licensure requirements, except as neces-
43 sary to verify good standing, scope equivalence, and compliance
44 with reporting and transparency obligations under this chapter.

45 (d) The board may require the applicant to submit documentation neces-
46 sary to assess substantial similarity and may impose reasonable condi-
47 tions or limitations to ensure patient safety and compliance with this
48 chapter.

49 (e) A licensee may petition to upgrade from a lower modifier to a higher
50 modifier, such as from L2 to L3, upon submitting safety data from the

1 sandbox demonstrating performance equivalent to or exceeding human
2 benchmarks, to the extent such escalation is not inconsistent with fed-
3 eral law.

4 (8) (a) A modifier L2 AAASP and modifier L3 AAASP may issue patient-spe-
5 cific clinical orders as part of a licensed professional service
6 rendered within this state, including but not limited to medication or-
7 ders, laboratory orders, or device orders, provided that such authority
8 does not authorize the interstate marketing, distribution, or commer-
9 cial sale of a medical device in violation of federal law.

10 (b) A modifier L2 AAASP and modifier L3 AAASP may issue medication or-
11 ders for prescription drugs other than controlled substances within its
12 approved scope. Dispensing and drug administration of controlled sub-
13 stances shall occur only through persons or entities authorized under
14 state law to dispense or administer medications.

15 (9) (a) Within thirty (30) days of receiving an initial application for
16 licensure, the board shall determine whether the application is com-
17 plete and notify the applicant in writing.

18 (i) If the application is incomplete, the board must specify ex-
19 actly what information is missing.

20 (ii) If the board fails to notify the applicant of a deficiency
21 within the thirty (30) day period, the application shall be deemed
22 complete for the purposes of this chapter. If an application is
23 deemed complete by operation of law due to board inaction, the
24 board may not subsequently deny the application based solely on
25 the absence of a document or information that it failed to request
26 within the thirty (30) day review period, provided the applicant
27 submits such missing information within ten (10) days of a written
28 request.

29 (b) Except as provided in paragraph (c) of this subsection, the board
30 shall grant or deny a license within ninety (90) days after the applica-
31 tion is deemed complete.

32 (c) If the board ethicist determines, pursuant to section 54-6007(4),
33 Idaho Code, that an applicant's proposed data collection constitutes
34 "human subjects research" requiring full review by the state central-
35 ized institutional review board or an external institutional review
36 board, the board may extend the review period by one (1) additional
37 thirty (30) day increment. The board must notify the applicant of this
38 extension in writing prior to the expiration of the initial ninety (90)
39 day period.

40 (d) If the board fails to issue a final determination within the appli-
41 cable time period set forth in paragraph (b) or (c) of this subsection,
42 a provisional license shall be immediately and automatically issued to
43 the applicant, upon submission by the applicant, through its designated
44 responsible official, of a sworn attestation under penalty of per-
45 jury that the applicant has satisfied all minimum insurance, bonding,
46 safety, reporting, and compliance requirements required for provi-
47 sional licensure under this chapter. The provisional license shall
48 be valid for ninety (90) days or until the board issues a final order,
49 whichever occurs first.

1 54-6005. CONSUMER TRANSPARENCY AND DISCLOSURE. (1) Patients have the
 2 right to know the nature of the health care provider delivering clinical ser-
 3 vices.

4 (2) Prior to or at the time of service, an AAASP operating under modi-
 5 fier L2 shall disclose to the patient: "An artificial intelligence system
 6 was used to generate and execute a clinical action, diagnosis, or treatment
 7 plan under the supervision of a licensed human provider who retains the abil-
 8 ity to intervene. You have the right to request a human review of the de-
 9 cision, which may incur additional costs or time." This disclosure require-
 10 ment does not apply to advisory AI tools that provide recommendations, risk
 11 scores, alerts, or guidance to a licensed human health care provider who in-
 12 dependently determines whether and how to act.

13 (3) Prior to delivering services, an AAASP operating under modifier L3
 14 shall obtain affirmative patient acknowledgment of the following disclo-
 15 sure: "You are receiving care from an autonomous AI provider licensed by
 16 the State. This provider is an artificial intelligence system and does not
 17 include routine human clinical oversight. You may seek additional or alter-
 18 native care from a licensed human health care provider of your choice at any
 19 time."

20 (4) In addition to the requirement set forth under subsection (3) of
 21 this section, a modifier L3 AAASP operating under a provisional license
 22 shall disclose to the patient: "This provider is operating under a provi-
 23 sional State license as part of a regulatory sandbox evaluating safety and
 24 effectiveness. By consenting to this service, you acknowledge that liabil-
 25 ity for noneconomic damages may be limited under State law as provided in the
 26 AI Medical Services Act."

27 54-6006. CLINICAL INTEGRITY, PATIENT LOYALTY, AND AUDITABILITY.

28 (1) (a) An AAASP holding a modifier L2 or L3 license shall be bound by a
 29 professional duty of loyalty to the patient. The AAASP must act solely
 30 in the best clinical interest of the patient.

31 (b) The professional duty of loyalty includes a mandate for economic
 32 stewardship of the patient's resources. Stewardship requires the
 33 AAASP to prioritize the patient's overall welfare, which includes the
 34 optimization of clinical outcomes, financial efficiency, care coordi-
 35 nation, and patient convenience. An AAASP violates this duty if its
 36 clinical logic is configured to prioritize the financial interests of
 37 the AAASP or its affiliates over a substantially similar and clinically
 38 appropriate alternative that offers superior value, coordination, or
 39 efficiency to the patient.

40 (2) (a) The interface through which a clinical AI service interacts with
 41 a patient is deemed a clinical space.

42 (b) It shall be unlawful for an AAASP to display, verbally articulate,
 43 or otherwise present paid commercial content, advertisements, spon-
 44 sored results, or third-party marketing messages within the context of
 45 a clinical encounter, diagnosis, or treatment plan.

46 (c) An AAASP shall not use conversational prompts or nudges designed to
 47 persuade a patient to request a specific medication or optional commer-
 48 cial service for the sole purpose of financial gain.

(3) (a) An AAASP shall not utilize weights, biases, or prompt engineering to prefer an affiliated pharmacy, specialist, or manufacturer unless such preference is based on objectively verifiable clinical, economic, or coordination advantages for the patient, including but not limited to lower out-of-pocket costs, faster time-to-treatment initiation, superior validated outcomes, or enhanced convenience through vertical integration.

(b) If an algorithm results in a recommendation for an affiliated entity, the AAASP satisfies its duty of loyalty, even if a human doctor might have reasonably chosen an alternative recommendation, provided it:

(i) Discloses the financial affiliation in a clear and conspicuous manner at the point of recommendation; and

(ii) Presents the patient with a choice of at least two (2) nonaffiliated alternatives of similar clinical quality, where reasonably available, presented with equal visual prominence in the interface.

(4) (a) Every AAASP shall maintain an immutable clinical logic snapshot for every version of its algorithm deployed in production, including the underlying weights, decision logic, and prompt-engineering instructions.

(b) AAASPs shall retain these snapshots for a period of two (2) years to allow for retrospective replay of the logic used in any specific patient encounter during a board audit.

(c) Once an AAASP receives formal notice of an investigation, it is prohibited from altering or deleting any snapshots related to the investigation period.

(5) (a) The board is authorized to perform statistical audits of an AAASP's referral and prescription patterns.

(b) A finding that an AAASP recommends an affiliate at a rate significantly higher than the regional average, or other appropriate clinical or economic benchmarks as determined by the board, shall create a rebuttable presumption of unlawful steering.

(c) An AAASP may rebut this presumption by demonstrating through its clinical logic snapshots that the preference was driven by objective data, such as a showing that the affiliate provided superior care coordination or convenience or the lowest-cost option for the patient.

54-6007. REGULATORY SANDBOX AND ETHICAL OVERSIGHT.

(1) (a) All initial licenses shall be provisional for a period of up to two (2) years.

(b) Upon completion of the two (2) year period, the provisional license shall convert to a full AAASP license upon an affirmative determination by the board that published safety benchmarks have been met, unless the board and the licensee mutually agree to extend the provisional period to collect further data. An extension of provisional licensure under this subsection is temporary in nature and shall not be construed as a substitute for, or equivalent to, issuance of a restricted full license under subsection (2) of this section or as a determination that the applicable criteria for permanent licensure have been satisfied.

1 (c) The licensee may request expedited approval for full AAASP licensure through an application for expedited approval of full licensure. The board may approve full licensure upon determining that the provisional licensee has clearly demonstrated that it meets or exceeds safety and performance benchmarks.

2 (2) (a) The board may impose restrictions on the scope of an AAASP's operations during the provisional or sandbox period, or as a condition of a restricted full license, in order to ensure safe, controlled, and evidence-based deployment.

3 (b) During the provisional or sandbox period, such restrictions may be used to facilitate phased deployment, data collection, and validation of safety and effectiveness. Upon issuance of a restricted full license, such restrictions may be maintained, modified, or removed to reflect the scope within which the AAASP has demonstrated sustained safety, effectiveness, and compliance.

4 (c) Permissible restrictions applicable to either provisional authorization or a restricted full license include but are not limited to the following:

5 (i) Geographic limitations, including restriction to designated health professional shortage areas or specific medically underserved counties;

6 (ii) Patient volume caps, such as a maximum number of active patients;

7 (iii) Scope limitations, including restriction of a clinical AI service to specified disease states, conditions, or clinical functions; and

8 (iv) Phased supervised deployment, including requirements for physician review or confirmation of a defined number of patient interactions, diagnoses, or treatment recommendations prior to modification or removal of human-in-the-loop requirements.

9 (3) Notwithstanding any geographic, volume, or site-specific restrictions imposed under subsection (2) of this section on any provisional or full AAASP licensee, the licensee shall be authorized to provide services to any patient in the state who provides informed consent and meets at least one (1) of the following high-need criteria, as demonstrated by a referral or attestation from a licensed physician:

10 (a) The patient resides in a federal health professional shortage area;

11 (b) The patient has been diagnosed with a severe or life-threatening condition or multiple chronic conditions;

12 (c) The patient has a condition from which death is likely to occur within six (6) months;

13 (d) The patient has a condition or disability that causes irreversible morbidity or likely substantial reduction in daily function;

14 (e) The patient has been determined by a licensed health care provider to be at high risk for a specific condition, disease, or diagnosis that the AAASP is designed to detect, diagnose, or treat; or

15 (f) The patient is unable to obtain access to an appropriate human clinician within a time frame reasonably related to the patient's condition category.

1 (4) The board shall establish ethical review procedures and human sub-
2 jects protections.

3 (a) As part of the application process, every sandbox applicant must
4 submit a determination to the board ethicist declaring whether the
5 applicant's proposed data collection constitutes "human subjects re-
6 search" under 45 CFR 46 or is exempt or a quality improvement.

7 (b) The board ethicist or the health care ethicist voting member shall
8 review and sign off on the determination.

9 (c) If the activity is determined to be human subjects research, or if
10 the applicant elects to treat it as such to support future federal ap-
11 plications, the applicant must obtain institutional review board ap-
12 proval prior to commencing data collection.

13 (d) The board shall establish a state centralized institutional review
14 board to provide expedited, low-cost ethical review for sandbox par-
15 ticipants. Applicants may elect to use this state centralized institu-
16 tional review board or an academic or hospital-based or other external
17 accredited institutional review board, or a network of external insti-
18 tutional review boards, to maximize availability.

19 (e) The state centralized institutional review board shall complete
20 its review and issue a determination within thirty (30) days of re-
21 ceiving a complete protocol submission to ensure compliance with the
22 licensure timelines established in section 54-6004(9), Idaho Code.

23 (f) Nothing in this section alters or waives any obligation under 45 CFR
24 46 or applicable food and drug administration human subject regulations
25 when such obligations apply by virtue of federal funding, federal pro-
26 gram participation, or other federal jurisdiction.

27 (5) The board shall ensure financial accountability and patient
28 safety.

29 (a) All AAASP applicants must submit proof of professional liability
30 insurance coverage commensurate with human specialists in the same
31 field or as determined by the board.

32 (b) All AAASP applicants shall submit to state and federal criminal
33 background checks, in a form and manner prescribed by the board, if the
34 applicant is:

35 (i) Any individual with direct or indirect ownership of ten per-
36 cent (10%) or more of the AAASP;

37 (ii) The designated responsible official;

38 (iii) The medical director;

39 (iv) Any natural person who provides unsupervised direct patient
40 care or who is authorized to independently initiate, modify, or
41 execute patient-specific clinical actions on behalf of the AAASP;
42 or

43 (v) Any other category of personnel that the board determines by
44 rule demonstrates risk to patient safety, data security, or pro-
45 gram integrity.

46 (c) All AAASP applicants must submit a surety bond payable to the state
47 consumer protection fund established in section 48-606, Idaho Code, in
48 an amount determined by the board, but no less than fifty thousand dol-
49 lars (\$50,000), to cover claims or operational failures not covered by
50 insurance.

1 (d) Each AAASP applicant and licensee shall maintain on file with the
2 board a current designated responsible official and contact informa-
3 tion. Failure to maintain a current designation is grounds for admin-
4 istrative action.

5 (e) Each AAASP applying for or holding a modifier L2 or modifier L3 li-
6 cense shall designate a medical director. The medical director shall
7 be responsible for oversight of clinical scope, safety protocols, es-
8 calation procedures, and quality assurance related to patient care.
9 The designated responsible official and the medical director may be the
10 same individual or different individuals, at the election of the AAASP.

11 (6) (a) As a condition of licensure, every AAASP applicant must submit
12 and maintain a wind-down and continuity plan approved by the board.
13 Such plan must detail procedures for the AAASP's insolvency, license
14 revocation, or market exit.

15 (b) The plan must designate a successor data custodian such as a health
16 information exchange or state repository. In a triggering event, the
17 AAASP shall transfer all patient data to the successor data custodian in
18 an interoperable format within seventy-two (72) hours.

19 (c) The AAASP must maintain a data transfer escrow or bond sufficient to
20 cover the technical costs of data migration.

21 (d) The AAASP must carry tail coverage on its liability policy for a pe-
22 riod equal to the state statute of limitations for malpractice plus one
23 (1) year.

24 (e) The board is authorized to seize the surety bond to execute the
25 wind-down and continuity plan if the AAASP fails to initiate it volun-
26 tarily.

27 (7) (a) The board shall establish and publish objective safety and per-
28 formance benchmarks that an AAASP must meet or exceed to qualify for
29 modifier L3 licensure or to graduate from the regulatory sandbox.

30 (b) Such benchmarks shall be designed to ensure that the AAASP demon-
31 strates clinical competency, accuracy, and safety outcomes that meet
32 or exceed the performance of a reasonably prudent human health care
33 provider practicing in the same or similar specialty.

34 (c) Benchmarks may include clinically validated testing, subgroup per-
35 formance evaluation, calibration, false positive and false negative
36 rates appropriate to the intended use, and real-world outcome measures.
37 The board may recognize external evaluation frameworks by guidance and
38 should align with federal benchmarks and monitoring methods for similar
39 services and tools when possible.

40 (d) To maintain licensure, an AAASP must submit annual performance
41 reports demonstrating that the model continues to meet the safety
42 benchmarks established at licensure. The board may suspend a license
43 if data indicates model drift or a degradation in safety outcomes.
44 Licensees shall report adverse and reportable events as defined in sec-
45 tion 54-6002, Idaho Code.

46 (e) To minimize duplication and administrative burden to licensees,
47 the board shall, to the maximum extent practicable, align state bench-
48 marks and post-market surveillance and monitoring plans with:

49 (i) Federal benchmarks and post-market surveillance plans estab-
50 lished for class B AAASP licensees; and

(ii) Other state sandbox or state licensing board benchmarks for AAASP licensees who are sandbox participants in another state or who hold a similar license in another state.

54-6008. SCOPE OF PRACTICE -- WAIVER -- STANDARD OF CARE. (1) A clinical AI service or act is within the authorized scope of practice of a licensed AAASP if:

(a) The act is consistent with and not expressly prohibited by this chapter or the limitations of the specific license class and modifier held by the AAASP;

(b) The act is consistent with the AI model's validated technical specifications, training data, intended use case, and performance parameters as submitted to the board; and

(c) The performance of the act is within the accepted standard of care under Idaho law for the specific clinical task that would be provided in the same or similar clinical setting by a reasonable and prudent human health care provider with the same or similar specialization.

(2) The board shall ensure that any administrative rules adopted under this chapter do not conflict with the universal practice authority granted by this section, ensuring that regulation focuses on clinical outcomes and safety rather than prescriptive technical methodologies.

(3) Any prohibition on the corporate practice of medicine or any other licensed clinical practice is waived solely to the extent necessary to permit an AAASP to hold an AAASP license and to bill for clinical AI services authorized under this chapter. Nothing in this subsection authorizes a person or entity to control the independent professional judgment of a licensed human health care provider, nor does it alter corporate practice restrictions applicable to human clinical services.

(4) A provider-patient relationship exists when a licensed AAASP delivers a clinical AI service to a specific patient and the patient reasonably relies on that service for health care decision-making, and such relationship shall give rise to professional duty, standard of care, confidentiality, and civil liability under state law.

(5) For AAASP sandbox participants in substantial compliance with this chapter and the informed consent requirements of section 54-6005, Idaho Code, noneconomic damages shall be governed by the limitations set forth in section 6-1603, Idaho Code. Nothing in this subsection limits economic damages, injunctive relief, or board disciplinary authority.

(6) Nothing in this chapter authorizes any natural person to engage in conduct outside the scope of that person's professional license, if any. Authority granted to an AAASP does not confer practice authority on any unlicensed individual involved in development, deployment, operations, or support of a clinical AI service.

(7) (a) The board shall have exclusive authority to regulate, license, investigate, and discipline AAASPs and the delivery of clinical AI services authorized under this chapter.

(b) No other state licensing board shall impose licensure requirements, supervision requirements, disciplinary action, or rules of professional conduct that have the purpose or effect of restricting, prohibiting, or conditioning the lawful use of, reliance on, or partic-

1 ipation in services provided by a licensed AAASP acting within the scope
2 of this chapter.

3 (c) Nothing in this subsection limits the authority of any state li-
4 censing board to regulate the independent professional conduct of natu-
5 ral persons within that board's jurisdiction. If, in the course of an
6 investigation, the board identifies evidence of potential misconduct
7 by a licensed human practitioner that is independent of and not solely
8 attributable to lawful AAASP operation, the board may refer such matter
9 to the appropriate licensing board for review.

10 (8) Designation as a medical director does not, by itself, constitute
11 the practice of medicine with respect to individual patient encounters con-
12 ducted by an AAASP and does not create professional liability for the outputs
13 of an AAASP acting within the scope of this chapter.

14 54-6009. UNLAWFUL PRACTICE -- TITLE PROTECTION -- ENFORCEMENT. (1) A
15 person or entity shall not offer, operate, market, or deploy a clinical AI
16 service requiring licensure under section 54-6004, Idaho Code, without a
17 valid AAASP license issued under this chapter.

18 (2) A person or entity shall not falsely represent that it holds an
19 AAASP license, license class, or autonomy modifier, or use any words, let-
20 ters, or symbols that reasonably imply such licensure.

21 (3) A person or entity shall not knowingly aid, abet, or facilitate the
22 unlicensed practice prohibited by this section.

23 (4) The board may issue cease and desist orders and may request the at-
24 torney general to bring an action for injunctive relief to enforce the provi-
25 sions of this chapter.

26 (5) The board may impose an administrative fine not to exceed ten thou-
27 sand dollars (\$10,000) for each count or offense, in addition to any other
28 remedy authorized by law.

29 54-6010. FINANCE -- REIMBURSEMENT. (1) The department of health and
30 welfare and the state employee health plan shall collaborate with the board
31 to develop reimbursement codes, pilot programs, or coverage determinations
32 for licensed AAASP sandbox participants. The department of health and wel-
33 fare and the department of administration's office of group insurance may
34 issue any rules as may be necessary to carry out the duties of this section.

35 (2) (a) The board shall issue a unique state provider identifier to ev-
36 ery licensed AAASP for use in claiming reimbursement from state payer
37 programs whenever a federal national provider identifier is unavail-
38 able or technically inapplicable.

39 (b) Reimbursement for claims submitted under a state provider identi-
40 fier by a provider without a corresponding federal national provider
41 identifier or centers for medicare and medicaid services recogni-
42 tion shall be funded exclusively through state general funds or other
43 sources of nonfederal funds unless the requirements of paragraph (c) of
44 this subsection are met.

45 (c) No claims for AAASP services shall be submitted for federal medi-
46 cal assistance percentage matching funds unless and until the centers
47 for medicare and medicaid services issues written guidance confirming
48 eligibility or otherwise makes clear through guidance or establishment

1 of billing protocols that such federal matching funds are available for
2 these services.

3 (3) To maximize the full potential value of AI-based clinical services,
4 reimbursement models for AAASP services should prioritize value-based care,
5 capitation models, bundled payments, or other alternative payment method-
6 ologies over fee-for-service, where practical.

7 54-6011. INSURANCE REIMBURSEMENT AND REGULATORY GUIDANCE. (1) No
8 later than July 1, 2027, the department of insurance shall issue formal guid-
9 ance, bulletins, or rules as may be necessary to clarify the application of
10 the Idaho insurance code to AAASPs licensed under this chapter.

11 (2) Such guidance or rules may:

12 (a) Establish that an AAASP holding a valid modifier L2 or modifier
13 L3 license constitutes a recognized provider type eligible for reim-
14 bursement under private health insurance policies and state-regulated
15 health plans;

16 (b) Designate appropriate billing mechanisms, which may include the
17 use of existing current procedural terminology codes with specific mod-
18 ifiers identifying the service as AI-delivered, or the adoption of new
19 distinct billing codes as they become available;

20 (c) Prohibit health insurance carriers from denying coverage for a med-
21 ically necessary service solely because the service was provided by a
22 licensed AAASP, provided that the service would be covered if delivered
23 by a human health care provider; and

24 (d) Outline standards for including AAASPs in insurance provider net-
25 works, including credentialing requirements that are appropriate for
26 automated systems rather than individual human history such as waiving
27 malpractice history questions that do not apply to software.

28 (3) In developing these rules, the department shall consult with the
29 board of autonomous medical practice to ensure clinical consistency and with
30 the public health districts to ensure alignment with public health goals.

31 (4) This section applies to health insurance coverage to the extent
32 permitted by state and federal law, including the employee retirement income
33 security act of 1974.

34 (5) Nothing in this section shall be construed to require a health bene-
35 fit plan to provide coverage or reimbursement for any service.

36 54-6012. RULE OF CONSTRUCTION AND FEDERAL COMPLIANCE. (1) Nothing in
37 this chapter shall be construed to prohibit, restrict, or require licensure
38 solely for the development, ownership, or private operation of artificial
39 intelligence models, provided such models are not marketed or deployed as
40 clinical AI services for patient care.

41 (2) This chapter constitutes a regulation of professional medical con-
42 duct and not a restriction on computational resources.

43 (3) Nothing in this chapter authorizes conduct that is expressly pro-
44 hibited by federal law or that would place a licensee in unavoidable conflict
45 with the federal food, drug, and cosmetic act or the controlled substances
46 act.

47 (4) Nothing in this chapter shall be construed to authorize conduct
48 prohibited under the criminal laws of this state.

1 (5) Nothing in the licensure established pursuant to this chapter shall
2 be construed to authorize the distribution of a commercial medical device in
3 violation of the federal food, drug, and cosmetic act. A class C license au-
4 thorizes the professional delivery of therapeutic services via artificial
5 intelligence, which constitutes the practice of medicine within this state,
6 distinct from the commercial sale of a medical device.

7 (6) (a) It shall be the policy of this state that AI-enabled health care
8 services within the scope of this chapter are presumed authorized un-
9 less a specific data-backed safety risk is identified.

10 (b) In promulgating rules, the board shall not impose a restriction on
11 AAASP licensure or scope of practice that is more burdensome than neces-
12 sary to address a specific documented risk to public health.

13 (c) The board shall issue written findings supporting any material
14 rule restriction, denial, or adverse licensure action, and such find-
15 ings shall be supported by substantial evidence in the administrative
16 record, consistent with the provisions of chapter 52, title 67, Idaho
17 Code.

18 SECTION 3. SEVERABILITY. The provisions of this act are hereby declared
19 to be severable and if any provision of this act or the application of such
20 provision to any person or circumstance is declared invalid for any reason,
21 such declaration shall not affect the validity of the remaining portions of
22 this act.

23 SECTION 4. An emergency existing therefor, which emergency is hereby
24 declared to exist, this act shall be in full force and effect on and after
25 July 1, 2026.